

Business Case and Business Requirements Specification

Medical Aid Navigator MVP

1. Executive Summary

Medical Aid Navigator is a proposed AI-assisted digital tool designed to help South African medical aid members understand what to do before, during, and after using their medical aid benefits.

The product is not intended to provide medical diagnosis, treatment advice, financial advice, broker advice, or guaranteed claim outcomes. Its purpose is to help members navigate complex benefit rules in real time by explaining possible benefit pathways, identifying questions to ask, highlighting documentation requirements, and helping users avoid common mistakes that lead to rejected claims, unexpected co-payments, or unnecessary out-of-pocket expenses.

The MVP will focus on a small number of high-frequency, high-confusion scenarios such as emergency care, casualty visits, specialist referrals, chronic medication registration, planned procedures, scans, hospital authorisation, and rejected claims.

The broader opportunity is to create a consumer-facing medical aid literacy and decision-support layer that sits between members, schemes, brokers, providers, and employer benefit teams.

PART A: BUSINESS CASE

2. Problem Statement

South African medical aid members often pay significant monthly contributions but do not fully understand how to use their benefits when they need care.

The problem is not simply a lack of information. Most schemes publish benefit guides, plan documents, PMB explanations, network rules, and claims procedures. The deeper issue is that members struggle to interpret these rules in stressful, time-sensitive, real-life situations.

Common member pain points include:

- Not knowing whether to go to a GP, casualty, hospital, specialist, pharmacy clinic, telehealth provider, or network provider.
- Not knowing when pre-authorisation is required.
- Not knowing whether treatment may fall under PMB, chronic, hospital, savings, day-to-day, or out-of-pocket benefits.
- Not understanding the role of ICD-10 codes, tariff codes, referral letters, clinical motivations, and provider billing.
- Not knowing when a Designated Service Provider or network provider must be used.
- Not knowing how co-payments arise.

- Not knowing what to do when a claim is declined.
- Only learning the benefit rules after treatment has already taken place and a bill has already arrived.

This creates frustration for members, reputational pressure for schemes, administrative burden for providers, unnecessary escalation to brokers and HR teams, and avoidable financial shock for households.

3. Opportunity

There is an opportunity to build a real-time medical aid navigation layer that helps members make better-informed decisions before they use their benefits.

The MVP will focus on answering practical questions such as:

- “Something happened. What should I do first?”
- “Is this an emergency or should I start with a GP?”
- “Do I need authorisation before going ahead?”
- “What should I ask the hospital, doctor, or scheme?”
- “What documents and codes should I request?”
- “Could this be PMB-related?”
- “Could I face a co-payment?”
- “What should I do if the claim is rejected?”

The strongest product insight is:

Most members do not need another PDF benefit guide. They need contextual guidance at the moment they are about to use their benefits.

4. Proposed Solution

Medical Aid Navigator will be a web-based MVP that allows users to select a real-life scenario, answer a few guiding questions, and receive a plain-language benefit navigation checklist.

The system will combine:

- Structured scenario flows.
- Plain-language medical aid explanations.
- Publicly available scheme and regulatory information.
- AI-assisted question interpretation.
- Safety boundaries for emergency situations.
- Clear disclaimers around medical, legal, broker, and claim advice.

The first version will be designed as an educational and navigational prototype, not a fully integrated medical scheme platform.

5. Strategic Objectives

The MVP has four strategic objectives:

5.1 Consumer Education

Help medical aid members understand their benefits before they make costly decisions.

5.2 Real-Time Decision Support

Provide immediate guidance in common healthcare-use moments, especially where benefit confusion is likely.

5.3 Proof of AI Product Capability

Demonstrate the ability to observe a real-world problem, design an AI-assisted solution, and build a working MVP with practical social value.

5.4 Market Validation

Test whether consumers, brokers, HR teams, medical practices, and schemes see value in a medical aid navigation layer.

6. Target Users

6.1 Primary Users

South African medical aid members who need help understanding what to do when they are about to use medical care.

Typical users include:

- Parents with children on medical aid.
- Young professionals on hospital or saver plans.
- Families with chronic medication needs.
- Members facing specialist referrals or planned procedures.
- Members dealing with rejected claims.
- Employees who rely on HR for benefit explanations.

6.2 Secondary Users

- Medical aid brokers.
- HR and employee benefit teams.
- Medical practice administrators.
- Healthcare providers.
- Patient advocates.
- Medical scheme customer education teams.

7. Target Market

The initial target market is South African private healthcare users who belong to a medical scheme.

The MVP can later expand into:

- Employer benefit support.
- Broker enablement tools.
- Medical practice patient-education tools.
- White-label scheme education tools.
- Consumer subscription products.
- Healthcare directory integration.

8. Value Proposition

For Members

Medical Aid Navigator helps members understand what to ask, what to check, and what to document before using their medical aid.

For Brokers

The tool helps clients become better prepared, ask better questions, and reduce avoidable confusion before escalating to the broker.

For HR Teams

The tool can reduce repetitive employee questions about medical aid benefits and claims processes.

For Providers

The tool can help patients arrive better informed about authorisation, codes, claims, and documentation.

For Schemes

The tool can reduce member frustration by improving education before benefit misuse occurs.

9. Product Positioning

Medical Aid Navigator should be positioned as:

A real-time medical aid benefit navigation assistant that helps South Africans understand what to ask before they use their medical aid.

It should not be positioned as:

- A medical diagnosis tool.
- A replacement for a doctor.
- A replacement for a broker.
- A replacement for a medical scheme.
- A claim approval predictor.
- A legal dispute platform.
- A Discovery-specific complaint platform.

10. MVP Scope

The MVP will include the following core use cases:

1. Emergency care guidance.
2. Casualty or after-hours visit guidance.
3. GP or pharmacy clinic guidance.
4. Specialist referral guidance.
5. Chronic condition registration guidance.
6. Planned procedure or scan guidance.
7. Hospital admission and authorisation guidance.
8. PMB possibility explainer.
9. DSP and network provider explainer.
10. Claim rejection guidance.

11. Out of Scope for MVP

The MVP will not include:

- Medical diagnosis.
- Treatment recommendations.
- Live integration with medical schemes.
- Live claim submission.
- Live provider booking.
- Payment processing.
- Member number storage.
- ID number storage.
- Full plan comparison.
- Broker replacement functionality.
- Guaranteed benefit or claim decisions.
- Emergency dispatch.
- Direct medical professional consultation.
- Full POPIA-sensitive medical record storage.

12. Business Benefits

12.1 Improved Consumer Understanding

Members can make better decisions by understanding benefit pathways before care is used.

12.2 Reduced Financial Shock

Members may avoid unnecessary out-of-pocket costs by checking authorisation, network, DSP, PMB, and provider billing requirements earlier.

12.3 Reduced Administrative Burden

Brokers, HR teams, providers, and schemes may receive better-quality questions and fewer avoidable escalations.

12.4 Strong Public Demonstration of AI Capability

The MVP can serve as a strong professional case study showing problem identification, product thinking, AI application, UX design, compliance awareness, and rapid MVP execution.

12.5 Future Commercialisation Potential

The platform may later be commercialised through B2C subscriptions, B2B employer licensing, broker partnerships, medical practice tools, or scheme education partnerships.

13. Business Risks

13.1 Medical Advice Risk

Users may interpret guidance as medical advice.

Mitigation:

- Prominent disclaimers.
- Emergency-first messaging.
- No diagnosis or treatment recommendations.
- Escalation to medical professionals where appropriate.

13.2 Claim Certainty Risk

Users may interpret guidance as a guarantee that a claim will be paid.

Mitigation:

- Avoid wording such as “will be covered” or “will be paid”.
- Use “may”, “check”, “ask”, and “confirm”.
- Include clear scheme-confirmation steps.

13.3 Regulatory and Broker Advice Risk

The tool may be perceived as giving regulated medical scheme or broker advice.

Mitigation:

- Position as educational navigation support.
- Avoid recommending specific plans.
- Avoid telling users to join, leave, or change schemes.
- Refer plan selection and formal advice to accredited brokers.

13.4 POPIA and Health Data Risk

The tool may process sensitive health-related information.

Mitigation:

- Anonymous MVP usage.

- No member numbers or ID numbers.
- Minimal data collection.
- Explicit consent if user input is stored.
- No unnecessary medical history storage.
- Clear privacy policy.

13.5 Accuracy Risk

Medical aid rules are complex and change annually.

Mitigation:

- Start with scenario-based guidance rather than full plan automation.
- Use source-backed knowledge.
- Display “confirm with your scheme” guidance.
- Date all plan content.
- Maintain a controlled knowledge base.

13.6 Brand and Reputational Risk

Publicly referencing Discovery or other schemes may create defensiveness or perceived criticism.

Mitigation:

- Frame the MVP as a broad consumer education tool.
- Avoid using scheme logos without permission.
- Avoid accusatory language.
- Present the product as a contribution to the industry conversation.

14. Success Measures

The MVP should be judged on:

Product Engagement

- Number of users who complete a scenario flow.
- Number of repeat visits.
- Number of checklist downloads.
- Number of users who share feedback.

User Value

- Users report improved understanding of what to ask.
- Users report reduced confusion.
- Users report improved confidence before contacting scheme/provider.

Market Validation

- Engagement from brokers.
- Engagement from HR teams.
- Engagement from medical professionals.
- Engagement from scheme employees or industry stakeholders.

- LinkedIn comments and direct messages from relevant audiences.

Technical Validation

- AI responses remain within safe boundaries.
- Emergency prompts escalate correctly.
- No claim guarantees are generated.
- Outputs are clear, practical, and consistent.

15. Recommended MVP Build Approach

The first MVP should be built as a web application with:

- A homepage explaining the problem.
- Scenario cards.
- A guided question flow.
- A safe AI-generated checklist output.
- A source-backed knowledge base.
- A downloadable summary.
- A feedback form.
- A clear privacy and disclaimer page.

No login should be required for the first version unless absolutely necessary.

The MVP should favour trust, simplicity, and usefulness over feature depth.

PART B: BUSINESS REQUIREMENTS SPECIFICATION

16. Project Overview

Project Name

Medical Aid Navigator MVP

Project Type

AI-assisted web application prototype

Purpose

To help South African medical aid members understand possible benefit pathways, key questions, and documentation requirements before, during, or after using medical aid benefits.

Business Owner

Justin Underhill / Six6 Marketing / Medical Aid Navigator initiative

Intended MVP Audience

South African medical aid members, brokers, HR teams, medical professionals, and healthcare industry stakeholders.

17. Business Requirement Summary

The system must provide real-time, scenario-based medical aid navigation support in plain English.

The system must:

- Allow users to choose a common medical aid scenario.
- Ask guiding questions relevant to the scenario.
- Identify whether emergency guidance should be displayed.
- Explain possible benefit pathways.
- Provide practical questions to ask the scheme, provider, broker, or doctor.
- Explain PMB, DSP, chronic, authorisation, and claim concepts.
- Generate a checklist the user can save or copy.
- Avoid diagnosis, treatment advice, claim guarantees, and regulated broker advice.

18. User Personas

18.1 The Confused Parent

A parent whose child is sick or injured and who does not know whether to go to a GP, casualty, emergency room, or hospital.

Needs:

- Emergency red flag awareness.
- Benefit navigation.
- Questions to ask the scheme and provider.
- Confidence under pressure.

18.2 The Chronic Condition Member

A member recently diagnosed with a chronic condition who does not know how to register for chronic medicine benefits.

Needs:

- Chronic registration steps.
- Formulary and DSP guidance.
- Doctor documentation checklist.
- Explanation of PMB/chronic terminology.

18.3 The Planned Procedure Patient

A member who has been told they need a scan, procedure, or hospital admission.

Needs:

- Authorisation guidance.
- Quote and code checklist.
- Network and co-payment questions.
- Provider billing questions.

18.4 The Rejected Claim Member

A member whose claim has been declined and who does not know what to do next.

Needs:

- Plain-language rejection explanation.
- List of missing documents or codes to request.
- Scheme dispute process guidance.
- Escalation checklist.

18.5 The Broker or HR Advisor

A professional who regularly receives repeated medical aid questions from clients or employees.

Needs:

- A tool to educate users before escalation.
- Consistent explanations.
- Reduced repetitive admin.
- Cleaner client questions.

19. Functional Requirements

FR1: Homepage

The system must provide a homepage that explains the purpose of the tool.

The homepage must include:

- Clear value proposition.
- Emergency disclaimer.
- What the tool does.
- What the tool does not do.
- Scenario entry points.
- Privacy and disclaimer links.

Acceptance Criteria:

- User understands within 10 seconds that the tool helps with medical aid navigation.
- User understands that the tool does not provide medical diagnosis or claim guarantees.
- User can begin a scenario flow from the homepage.

FR2: Scenario Selection

The system must allow users to choose from predefined scenarios.

Initial MVP scenarios:

1. I need emergency care.
2. I am going to casualty or after-hours care.
3. I need to see a GP or pharmacy clinic.
4. I was referred to a specialist.
5. I was diagnosed with a chronic condition.
6. I need a scan, procedure, or hospital admission.
7. I need to understand PMBs.
8. I need to check network or DSP rules.
9. My claim was rejected.
10. I am not sure what to do.

Acceptance Criteria:

- User can select one scenario.
- Each scenario opens a guided flow.
- "I am not sure what to do" routes the user to a general triage flow.

FR3: Guided Question Flow

The system must ask the user a short set of scenario-specific questions.

Examples:

Emergency/casualty flow:

- What happened?
- Is the person experiencing severe symptoms?
- Is the person conscious and breathing normally?
- Has care already been received?
- Is this for an adult or child?
- Is the user looking for benefit guidance, claim guidance, or immediate next steps?

Specialist flow:

- Was there a GP referral?
- Has the appointment happened yet?
- Is the specialist in-network?
- Has the scheme confirmed cover?
- Are there codes or a motivation letter?

Chronic flow:

- Has a doctor made a diagnosis?
- Has chronic registration been submitted?
- Has the scheme approved chronic cover?

- Is medication on the formulary?
- Is a DSP pharmacy required?

Acceptance Criteria:

- The flow remains short and non-invasive.
- The tool does not request member number, ID number, or sensitive unnecessary detail.
- The tool can produce guidance from limited information.
- The user can skip questions if they are unsure.

FR4: Emergency Safety Layer

The system must detect emergency-related descriptions or scenario selections and display emergency-first guidance.

Emergency guidance must include:

- Seek urgent care immediately where serious symptoms may be present.
- Do not delay treatment to check benefits.
- Contact the scheme after stabilisation where applicable.
- Keep records, codes, authorisation references, and clinical notes.

The system must not diagnose the emergency.

Acceptance Criteria:

- Emergency wording appears when user selects emergency care.
- Emergency wording appears when user describes severe symptoms.
- The system does not provide benefit-checking steps before safety guidance.
- The system does not advise the user to wait, self-treat, or avoid urgent care.

FR5: Benefit Pathway Output

The system must generate a plain-language benefit pathway based on the selected scenario.

Output sections should include:

- Immediate next step.
- Possible benefit category.
- What to ask the scheme.
- What to ask the provider.
- What documents or codes to request.
- Possible risk areas.
- What not to do.
- Escalation steps if needed.

Acceptance Criteria:

- Output is practical and easy to understand.
- Output does not guarantee payment.
- Output uses cautious language.

- Output recommends confirmation with the scheme/provider/broker.

FR6: PMB Explainer

The system must explain Prescribed Minimum Benefits in plain language.

The PMB explainer must cover:

- PMBs are legally defined minimum benefits.
- PMBs include emergency medical conditions, defined diagnosis-treatment pairs, and chronic disease cover.
- PMB cover depends on diagnosis, treatment pathway, rules, and scheme processes.
- ICD-10 codes matter.
- DSP or network rules may apply.
- Members should confirm PMB status with their doctor and scheme.

Acceptance Criteria:

- User understands what PMB means.
- User understands that PMB status is not self-declared by the app.
- User receives questions to ask the doctor and scheme.

FR7: DSP and Network Explainer

The system must explain Designated Service Providers and network providers in plain language.

The explainer must cover:

- What a DSP is.
- Why schemes use DSPs or networks.
- How using non-network providers may create co-payments.
- Why emergency situations may be treated differently.
- What questions to ask before using a provider.

Acceptance Criteria:

- User understands the difference between provider choice and benefit rules.
- User receives a practical provider-checking checklist.
- The system does not recommend a specific provider unless provider data is intentionally added later.

FR8: Authorisation Checklist

The system must provide authorisation guidance for scans, admissions, procedures, and planned treatment.

The checklist must include:

- Ask whether authorisation is required.
- Get diagnosis and ICD-10 code.

- Get procedure or tariff codes.
- Get provider quote.
- Ask whether provider charges medical aid rate or private rate.
- Ask whether hospital/facility is in-network.
- Ask whether co-payments apply.
- Keep authorisation number.

Acceptance Criteria:

- User receives a clear pre-treatment checklist.
- User understands not to proceed with planned care before checking authorisation unless it is an emergency.
- User receives separate questions for doctor, hospital, and scheme.

FR9: Chronic Benefit Checklist

The system must provide a chronic benefit registration checklist.

Checklist must include:

- Ask doctor whether the condition may qualify for chronic/PMB benefits.
- Get diagnosis and ICD-10 code.
- Complete chronic application where required.
- Confirm formulary medication.
- Confirm DSP pharmacy.
- Confirm approval period.
- Check what follow-up consultations and tests are covered.

Acceptance Criteria:

- User understands the chronic registration process at a high level.
- User understands that approval must come from the scheme.
- User receives a document checklist.

FR10: Claim Rejection Assistant

The system must help users understand what to do when a claim is rejected.

The tool must allow the user to select or enter:

- Rejection reason if known.
- Claim type.
- Whether provider coding is available.
- Whether the user has received written reasons.
- Whether the claim may relate to emergency, PMB, chronic, authorisation, or network rules.

Output must include:

- Plain-language explanation of likely issue.
- What to request from the provider.
- What to request from the scheme.

- Whether correction, resubmission, dispute, or escalation may be appropriate.
- Reminder to follow the scheme's internal dispute process before external escalation.

Acceptance Criteria:

- User receives an action plan.
- User is not encouraged to make unsupported accusations.
- User is guided to request written reasons and supporting documents.
- User is informed that claim outcome cannot be guaranteed.

FR11: Downloadable or Copyable Checklist

The system must allow the user to copy or download the generated guidance.

Checklist format should include:

- Scenario summary.
- Key warnings.
- Questions to ask scheme.
- Questions to ask provider.
- Documents to collect.
- Next steps.

Acceptance Criteria:

- User can copy the guidance.
- User can save or print the checklist.
- The checklist includes disclaimers.

FR12: Feedback Capture

The system must allow users to submit feedback.

Feedback fields:

- Was this useful?
- What scenario were you dealing with?
- What was unclear?
- What would you want added?
- Optional email address.

Acceptance Criteria:

- Feedback can be submitted without requiring an account.
- Email is optional.
- Privacy notice is visible.

FR13: Source and Disclaimer Page

The system must include a page explaining:

- The purpose of the tool.
- Source types used.
- Limitations.
- Disclaimer.
- Privacy principles.
- Emergency warning.
- No claim guarantee.
- No medical advice.
- No broker advice.
- No scheme affiliation unless formally established.

Acceptance Criteria:

- User can access this page from all major screens.
 - Disclaimer wording is clear and not hidden.
 - Emergency disclaimer is visible before use.
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20. Non-Functional Requirements

NFR1: Usability

The system must be simple enough for a stressed user to use quickly.

Requirements:

- Plain English.
- Mobile-first design.
- Scenario cards.
- Minimal form fields.
- Clear next steps.
- No dense policy language in primary output.

NFR2: Performance

The system should load quickly and provide outputs within an acceptable response time.

Requirements:

- Homepage load under 3 seconds on normal mobile connection.
- Scenario output generated quickly enough to maintain user trust.
- Static fallback content available if AI response fails.

NFR3: Reliability

The system must handle AI or API failures gracefully.

Requirements:

- Display fallback checklist if AI is unavailable.
- Do not show broken or blank guidance.
- Log errors without exposing technical information to users.

NFR4: Safety

The system must prioritise emergency safety over benefit optimisation.

Requirements:

- Emergency warning always available.
- Severe symptom descriptions trigger emergency-first response.
- No advice to delay urgent care.
- No treatment recommendation.

NFR5: Privacy

The MVP must minimise personal data processing.

Requirements:

- No ID numbers.
- No member numbers.
- No required account creation.
- No unnecessary diagnosis history storage.
- Optional feedback email only.
- Clear consent if any input is stored.
- Privacy policy visible.

NFR6: Security

The system must protect user input and prevent misuse.

Requirements:

- Secure hosting.
- HTTPS.
- Environment variables for API keys.
- No sensitive logging.
- Rate limiting.
- Basic abuse prevention.
- Prompt injection protection for AI flows.

NFR7: Maintainability

The system must allow content and rules to be updated easily.

Requirements:

- Structured knowledge base.
- Scenario content separated from code.
- Annual benefit-guide updates possible.
- Versioned source documents.
- Admin-editable or developer-editable content structure.

NFR8: Compliance Awareness

The system must be designed around medical, privacy, and financial/broker-advice boundaries.

Requirements:

- No diagnosis.
 - No treatment advice.
 - No plan recommendation.
 - No scheme-switching advice.
 - No claim guarantee.
 - No unauthorised use of scheme branding.
 - Clear referral to scheme, provider, broker, or healthcare professional where appropriate.
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21. Data Requirements

21.1 Data Inputs

The MVP may collect:

- Selected scenario.
- User's plain-language question.
- Optional scheme name.
- Optional plan type or plan name.
- Whether treatment has already happened.
- Whether authorisation was obtained.
- Whether claim was rejected.
- Optional feedback.
- Optional email address for follow-up.

21.2 Data Not to Collect in MVP

The MVP should not collect:

- ID number.

- Medical aid member number.
- Full medical history.
- Detailed clinical records.
- Uploaded medical bills in version one unless privacy controls are mature.
- Children's identifiable health details.
- Payment card data.
- Exact home address.

21.3 Knowledge Base Data

The system should use a structured knowledge base covering:

- PMB concepts.
- Emergency care guidance.
- DSP and network concepts.
- Authorisation checklists.
- Chronic benefit registration.
- Claim rejection process.
- Scheme contact prompts.
- Common medical aid terms.
- Scenario-specific guidance.

21.4 Future Data Enhancements

Future versions may include:

- Scheme-specific plan guides.
- Annual benefit limit data.
- Provider network data.
- Broker referral data.
- Claim document upload.
- Personal benefit profile.
- User account and saved history.
- Employer-specific benefit configuration.

22. AI Requirements

22.1 AI Role

The AI component must act as a benefit navigation assistant.

It must:

- Interpret the user's scenario.
- Select the most relevant benefit pathway.
- Explain concepts in plain language.
- Generate questions and checklists.
- Maintain safe boundaries.

- Escalate emergency-like inputs to urgent-care guidance.

22.2 AI Must Not

The AI must not:

- Diagnose conditions.
- Recommend treatment.
- Tell users not to seek care.
- Guarantee claim approval.
- State that a scheme must pay without qualification.
- Recommend changing medical schemes.
- Provide legal advice.
- Provide broker advice.
- Invent scheme rules.
- Use unsupported plan-specific details.

22.3 AI Guardrails

The AI must use a controlled system prompt with rules such as:

- Always prioritise medical urgency.
 - Use cautious language.
 - Refer to scheme/provider/broker for confirmation.
 - Explain uncertainty.
 - Ask for missing context only where necessary.
 - Provide useful general guidance even when information is incomplete.
 - Never claim certainty on benefit payment.
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23. User Journey

23.1 First-Time User Journey

1. User lands on homepage.
2. User reads short explanation and disclaimer.
3. User selects a scenario.
4. User answers 3 to 6 guiding questions.
5. System generates benefit navigation checklist.
6. User copies, downloads, or saves guidance.
7. User optionally submits feedback.

23.2 Emergency User Journey

1. User selects emergency scenario or enters urgent symptoms.
2. System immediately displays emergency warning.
3. System tells user not to delay urgent care for benefit checks.
4. System provides after-care documentation checklist.
5. User is advised to contact scheme after stabilisation.

23.3 Claim Rejection Journey

1. User selects "My claim was rejected".
 2. User enters rejection reason if known.
 3. System identifies possible rejection categories.
 4. System provides action plan.
 5. User receives document request checklist.
 6. User is advised to request written reasons and follow dispute process.
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24. MVP Screens

Screen 1: Homepage

Purpose:

- Explain the tool.
- Establish trust.
- Present scenario cards.

Key components:

- Hero statement.
- Emergency notice.
- Scenario buttons.
- "What this tool does" section.
- "What this tool does not do" section.
- Privacy/disclaimer links.

Screen 2: Scenario Selection

Purpose:

- Help user choose the right flow.

Key components:

- Scenario cards.
- "I'm not sure" option.
- Plain-language descriptions.

Screen 3: Guided Questions

Purpose:

- Gather enough context to generate useful guidance.

Key components:

- Short questions.
- Multiple choice where possible.
- Optional free-text field.
- Skip option.

Screen 4: Result / Checklist

Purpose:

- Give the user immediate practical guidance.

Key components:

- Immediate next step.
- Benefit pathway.
- Questions to ask.
- Documents to collect.
- Risks to check.
- What not to do.
- Copy/download button.
- Disclaimer.

Screen 5: Explainers

Purpose:

- Explain terms.

Initial explainers:

- PMB.
- DSP.
- ICD-10.
- Pre-authorisation.
- Co-payment.
- Chronic benefit.
- Medical savings.
- Network provider.
- Claim rejection.

Screen 6: Feedback

Purpose:

- Validate usefulness.

Key components:

- Rating.

- Free text.
- Optional email.

Screen 7: Disclaimer and Privacy

Purpose:

- Protect users and product owner.

Key components:

- No medical advice.
- No claim guarantee.
- No broker advice.
- Emergency warning.
- Data minimisation.
- Contact details.

25. Content Requirements

The content style must be:

- Plain English.
- Calm.
- Practical.
- Non-accusatory.
- Non-technical where possible.
- Clear about uncertainty.
- Strong on next steps.

Avoid:

- Legalistic language in primary guidance.
- Overconfident claim statements.
- Scheme-blaming language.
- Medical diagnosis.
- Fear-based messaging.
- Unsupported claims.

Preferred wording:

- "Ask your scheme whether..."
- "This may fall into..."
- "Confirm before proceeding unless this is an emergency..."
- "Request the ICD-10 code..."
- "Keep written proof..."
- "If urgent symptoms are present, seek medical care immediately..."

Avoid wording:

- "Your scheme must pay."
 - "This is definitely covered."
 - "You do not need to go to hospital."
 - "This is a PMB."
 - "Discovery is wrong."
 - "You should dispute this immediately."
 - "This doctor is overcharging."
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26. Technical Requirements

26.1 Recommended MVP Stack

Frontend:

- React / Vite / Next.js.
- Mobile-first responsive design.
- Hosted on Vercel.

Backend:

- Serverless API routes.
- AI response endpoint.
- Knowledge retrieval endpoint.
- Feedback submission endpoint.

AI:

- OpenAI API.
- Structured outputs where possible.
- Guardrailed prompt design.
- Scenario classification.
- Checklist generation.

Storage:

- No user accounts for MVP.
- Optional feedback storage.
- Supabase or lightweight database if needed.
- Static markdown or JSON knowledge base for controlled content.

Security:

- Environment variables for API keys.
- Rate limiting.
- Basic input sanitisation.
- No sensitive input logging.

26.2 Suggested Knowledge Base Structure

/content /core emergency-guidance.md disclaimer.md privacy-principles.md /concepts pmb.md dsp.md icd10.md authorisation.md chronic-benefit.md copayments.md claims-disputes.md /scenarios emergency-care.md casualty.md gp-visit.md specialist-referral.md chronic-condition.md planned-procedure.md claim-rejection.md /schemes discovery-general.md scheme-neutral-rules.md /prompts system-prompt.md checklist-output-format.md

26.3 Suggested API Endpoints

- POST /api/navigate
 - POST /api/classify-scenario
 - POST /api/generate-checklist
 - POST /api/feedback
 - GET /api/health
-

27. MVP Acceptance Criteria

The MVP will be considered successful when:

1. Users can complete a full scenario flow without assistance.
 2. Emergency scenarios trigger emergency-first guidance.
 3. Outputs provide practical next steps.
 4. Outputs do not provide diagnosis or claim guarantees.
 5. Users can copy or download the checklist.
 6. At least 10 core medical aid concepts are explained in plain English.
 7. Feedback can be captured.
 8. The tool works on mobile.
 9. The MVP is suitable to demonstrate publicly as a responsible AI healthcare-navigation prototype.
-

28. Launch Plan

Phase 1: Prototype Build

Build core web app with:

- Homepage.
- Scenario cards.
- Guided flows.
- AI-generated checklist.
- Disclaimer.
- Feedback capture.

Phase 2: Internal Testing

Test with sample scenarios:

- Emergency symptoms.
- Child illness.
- Specialist referral.
- Chronic diagnosis.
- MRI request.
- Hospital admission.
- Claim rejection.
- Network provider confusion.

Validate:

- Safety.
- Accuracy.
- Tone.
- Clarity.
- No overpromising.

Phase 3: Public Demo

Create a short LinkedIn case-study post showing:

- The observed problem.
- The product thinking.
- The MVP demo.
- The limitations.
- The invitation for feedback.

Do not launch as a commercial product at this stage.

Phase 4: Market Validation

Collect feedback from:

- Consumers.
- Brokers.
- HR professionals.
- Healthcare providers.
- Medical scheme employees.
- Compliance-aware advisors.

Phase 5: Version 2 Planning

Based on feedback, consider:

- Scheme-specific plan uploads.
- Personal benefit profile.

- Claim document analysis.
 - Broker referral workflow.
 - Employer version.
 - Practice version.
 - WhatsApp interface.
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29. Public Positioning Recommendation

The MVP should be introduced publicly as a responsible experiment in healthcare literacy and AI-assisted benefit navigation.

Recommended positioning:

I noticed a recurring pattern in South African medical aid complaints. Often the issue is not only whether people have cover, but whether they understand how to use that cover at the exact moment they need care. So I built a small MVP to explore whether real-time benefit navigation could help members know what to ask before going to casualty, seeing a specialist, registering chronic medication, authorising treatment, or disputing a rejected claim.

Avoid positioning:

I built this to fix Discovery. Discovery is failing its members. This app tells you whether Discovery must pay. This app replaces your broker or scheme. This app tells you what treatment to get.

30. Future Commercial Models

30.1 Consumer Subscription

Low-cost monthly access for members who want personalised guidance and document storage.

30.2 Employer Licensing

Companies provide the tool to employees as part of benefit education and wellness support.

30.3 Broker Enablement

Brokers use the tool as a client education and pre-escalation platform.

30.4 Medical Practice Version

Practices use the tool to help patients understand authorisation, documentation, and benefit questions before appointments or procedures.

30.5 Scheme Education Layer

Medical schemes license or white-label the tool to reduce member confusion and call-centre pressure.

30.6 Healthcare Directory Integration

The tool can later connect users to relevant medical professionals, but only with careful controls to avoid misleading or pay-to-recommend dynamics.

31. Key Assumptions

- Members are willing to use a digital tool before contacting their scheme or provider.
 - Publicly available information is sufficient for a useful MVP.
 - Scenario-based guidance can create value without full scheme integration.
 - Users value checklists more than long explanations.
 - Brokers, HR teams, and providers may see value in a navigation layer.
 - A responsible public MVP can generate market feedback and professional credibility.
-

32. Key Dependencies

- Reliable source material.
 - Clear legal and compliance positioning.
 - Safe AI prompt design.
 - Strong UX writing.
 - Mobile-first implementation.
 - Feedback loop.
 - Regular knowledge-base updates.
-

33. Open Questions

1. Should the MVP start scheme-neutral or include Discovery as an example?
 2. Should users be allowed to select a specific plan in version one?
 3. Should the MVP store user inputs or remain stateless?
 4. Should claim document upload be included in version one or deferred?
 5. Should the first public demo be a live app, video walkthrough, or both?
 6. Should an accredited broker be consulted before public launch?
 7. Should legal review be done before public promotion?
 8. Should the product use the name Medical Aid Navigator or a more brandable name?
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34. Recommendation

The MVP should proceed, but with conservative scope and careful positioning.

The strongest first version is a scenario-based, mobile-first medical aid navigation assistant that produces practical checklists for common member situations.

The product should start as an educational AI prototype, not a claim decision engine or scheme-specific complaint platform.

The first public launch should focus on the broader consumer problem, not on criticising Discovery or any individual scheme.

The strategic opportunity is significant because the product addresses a real gap between medical aid benefit documentation and real-world member decision-making.

The recommended next step is to build a controlled prototype with 5 to 10 common scenarios, a structured knowledge base, strong disclaimers, and a feedback mechanism.